

Feature	PICA (Lateral Medulla)	AICA (Lateral Pons)	ASA (Medial Medulla)
Face pain/temp	Ipsilateral ↓	Ipsilateral ↓	Normal
Body pain/temp	Contralateral ↓	Contralateral ↓	Normal
Motor	Normal	Normal	Contralateral paralysis
Key CN	Nucleus ambiguus	Facial nerve	Hypoglossal
Signature	Dysphagia	Facial paralysis	Tongue deviation

The way to actually remember under pressure

Don't memorize lists. Use this decision tree:

1. **Is it lateral or medial?**
 - Crossed findings → **lateral**
 - Pure motor → **medial**
2. **If lateral:**
 - Face droop? → **AICA**
 - Dysphagia/hoarseness? → **PICA**
3. **If medial:**
 - Think **ASA**

One-line mental model

Spinothalamic already crossed → body flips sides
Trigeminal hasn't crossed yet → face stays same side

PICA — Lateral Medullary (Wallenberg)

Blood supply: PICA

Location: Lateral medulla

What it hits:

- Spinothalamic → **contralateral body pain/temp ↓**
- Trigeminal → **ipsilateral face pain/temp ↓**
- Nucleus ambiguus → **dysphagia, hoarseness (KEY giveaway)**
- Vestibular → vertigo

- Inferior cerebellar peduncle → ataxia
- Sympathetics → **ipsilateral Horner**

Pattern:

- Face = same side
- Body = opposite side

👉 **"Don't PICA horse that can't eat"** (nucleus ambiguus)

 AICA — Lateral Pontine

Blood supply: AICA

Location: Lateral pons

What it hits:

- Same as PICA **BUT**
- Facial nerve (CN VII) → **facial paralysis**
- ↓ lacrimation, salivation
- ↓ taste (anterior 2/3)
- ± hearing loss (labyrinthine artery)

Pattern:

- Same **crossed pain/temp** as PICA

👉 **"Facial droop = AICA"**

 ASA — Medial Medullary

Blood supply: Anterior spinal artery

Location: Medial medulla

What it hits:

- Corticospinal → **contralateral paralysis**
- DCML → **contralateral vibration/proprio ↓**
- Hypoglossal → **tongue deviates TOWARD lesion**

Pattern:

- NO face pain/temp
- Pure motor + dorsal column

👉 "Medial = motor problem"